INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Capecitabine

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00216
Version 8

Twice daily on Days 1–14, then seven days off; each cycle is 21 days. Adjuvant colon or residual triple-negative breast treatment is eight cycles (about six months). Advanced-disease treatment continues while helping and tolerated.

Treatment stages

01 **21-day tablet cycle**
Oral chemotherapy

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

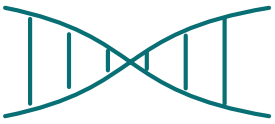
How the treatment works

Capecitabine is chemotherapy taken as tablets. NCCP 00216 covers several colorectal and breast cancer settings, including treatment after surgery and treatment of advanced disease. Ask which aim and duration apply to you.

ORAL CHEMOTHERAPY

Capecitabine

Xeloda



Becomes fluorouracil in the body

After you swallow capecitabine, your body changes it into fluorouracil. This interferes with the deoxyribonucleic acid (the cell's genetic material) (DNA) and RNA processes cells use to grow. Normal cells can also be affected, which helps explain mouth soreness, bowel symptoms and changes in blood counts.

Swallowed tablet → Converted to fluorouracil → Cell growth disrupted

What to watch for

Stop tablets and contact the team for significant diarrhoea, painful mouth ulcers, hand-foot soreness affecting activities, fever or dehydration. Chest pain needs urgent assessment. Do not restart until instructed.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and duration

The same medicine can be used for different purposes.

How long?

NCCP recommends six months / eight cycles for adjuvant colon or residual triple-negative breast cancer. For advanced disease, duration depends on response and tolerability.

Alternatives and decisions

Discuss the expected benefit, burdens, reasonable alternatives and the option of supportive care. The best choice depends on your cancer, fitness and preferences.

Evidence depends on the cancer setting

The X-ACT comparison concerns stage III colon cancer after surgery. A separate study, CREATE-X, tested adding capecitabine after surgery for HER2-negative breast cancer with residual invasive disease after chemotherapy, including a triple-negative subgroup. These studies address different situations and cannot give one benefit estimate for every person taking capecitabine.



When treatment is outside the medicine licence

NCCP identifies some uses of capecitabine as outside its licence in Ireland: stage II colon cancer, colorectal cancer that has spread but has then been completely removed, and residual triple-negative breast cancer after chemotherapy. If one applies to you, your team should explain this, the evidence and alternatives as part of consent.

Understanding the evidence

Ask which evidence matches your cancer, treatment aim and medicines.

In stage III colon cancer after surgery, capecitabine was at least as effective as the intravenous fluorouracil schedule studied. This comparison does not measure benefit against no treatment, and does not estimate benefit for breast cancer or cancer that has spread.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

How the treatment fits together

Capecitabine

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ORAL CHEMOTHERAPY

21-day tablet cycle

D1 D14 D15 D21

capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

The treating team confirms the aim, duration and next steps. The visual pattern does not replace your prescription.

BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00216 v8 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Capecitabine: treatment and checks

Colorectal and breast cancer

capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00216	Baseline bloods; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00216	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Tablets / hands and feet NCCP 00216	Confirm written dose, tablet supply and hold instructions	Review adherence, mouth, bowel and skin symptoms	Days 1–14 tablets; Days 15–21 planned break unless team changes it	Do not restart held tablets or replace missed doses without advice	Return unused tablets as instructed; follow persistent symptoms
Heart checks NCCP 00216	Review heart history. The team may arrange an electrocardiogram (ECG) or other tests if clinically indicated; NCCP does not mandate a routine ECG or serial echocardiogram schedule.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Response / scans NCCP 00216	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment

What each check is for

1



21-day tablet cycle

Oral chemotherapy

Capecitabine is chemotherapy taken as tablets. NCCP 00216 covers several colorectal and breast cancer settings, including treatment after surgery and treatment of advanced disease. Ask which aim and duration apply to you.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. DPD testing is required before starting the fluoropyrimidine unless already completed.

STEP 2

Taking the tablets

Use the prescribed morning and evening dose within 30 minutes after food on the agreed treatment days. Do not make up a missed or vomited dose. Follow the team's hold instructions for significant diarrhoea, mouth soreness or painful hands and feet, and contact them promptly.

STEP 3

If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.

STEP 4

Getting help between visits

Follow your oncology unit's urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

Tests and checks

Monitoring helps the team decide whether treatment can continue.

DPD testing

DPD helps break down fluoropyrimidine medicines. Low activity can cause severe toxicity; testing before treatment helps guide the plan but cannot prevent every serious reaction.

Blood, kidney and liver results

Counts, kidney and liver function are checked before each cycle. Diarrhoea and poor intake can worsen kidney function and make tablets harder to tolerate.

Your symptoms matter

Report bowel frequency, stoma output, mouth soreness, hand-foot pain, chest symptoms and what daily activities you can no longer manage.

Additional checks

The team sets the scan or follow-up plan for your particular treatment aim.

Extra checks when needed

Scans and tumour-marker tests follow your cancer-specific plan; there is no single timetable for everyone taking these tablets. If you take warfarin or another vitamin K antagonist, confirm who will check your international normalised ratio (blood-clotting test) (INR) and how often. Dehydration can worsen kidney function, so new diarrhoea, vomiting or poor drinking may require earlier blood tests.

Review, recovery and follow-up

Your plan can change as treatment progresses.

Dose changes are individual

A pause or reduction may be needed because of symptoms, blood results or kidney function. The team considers the treatment aim and other options.

After the last dose

Keep reporting new or persistent symptoms after treatment ends. Your team confirms follow-up, scans and recovery support; the timeline is not a forecast.

Prepare for review

Bring questions, symptoms and missed-dose records. Tell the team what matters most to you and ask them to explain the next decision.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Capecitabine tablets	 **Days 1–14** Twice daily as prescribed
Tablet-free days	 **Days 15–21** No planned capecitabine
Bowel / mouth / skin	 **Days 1–21** May develop or accumulate; report symptoms
Infection / feeling unwell	 **Days 1–21** Seek advice on any day

Your written plan comes first: capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Act when symptoms occur: Do not wait for the next visit or use the chart to decide a symptom is safe.

Symptoms can build: Bowel, mouth and hand-foot symptoms may accumulate and can continue during tablet-free days.

Across the course: The pattern can change between cycles. Dose changes and treatment breaks are made with the team.

> **Why these bands stay broad:** Treatment bands show the NCCP schedule. Symptom bands are broad teaching illustrations: they do not represent measured daily risks or predict when you will feel well. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00216 v8 · official regimen ↗ · eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule) ↗

> **There are no guaranteed "safe days".** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1–14: Treatment days

capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription. Use the current tablet label. Swallow whole with water with or within 30 minutes of food; do not crush or cut tablets.

Day 15–21: Planned tablet-free days

Do not take leftover tablets to replace doses missed earlier. Side effects may continue during this interval. Keep the oncology contact plan available and confirm the next review before restarting.

Support around the medicines

1 **Nausea prevention and rescue**

The team agrees a rescue medicine if needed; routine daily anti-sickness prevention is not automatically required for this oral low-risk schedule.

2 **Bowel symptoms**

Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.

3 **Counts and recovery**

White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.

4 **Skin and mouth care**

Use gentle skin and mouth care. Painful hands and feet or mouth sores can require a tablet hold and team review; do not continue simply to finish the pack.

Taking capecitabine safely

Tablets are active chemotherapy, with a specific schedule.

Meals and doses

Use your current prescription. Swallow tablets whole with water with or within 30 minutes after food. Do not cut, chew or crush them; ask pharmacy if swallowing is difficult.

Missed doses or vomiting

Do not replace a missed or vomited dose. Never double the next dose. Contact the team for repeated vomiting or uncertainty about the schedule.

Pause and restart

Stop tablets for significant diarrhoea, painful mouth/hand-foot symptoms affecting activity, fever or dehydration and seek advice. A toxicity-related pause requires instructions before restart.

Different tablet strengths

Different strengths may be combined to make your dose. A reduction changes tablet numbers: use the new label and do not calculate a change from this page.

Recognise symptoms and act

Do not wait for the next appointment if you feel unwell.

Bowel and hydration

Report diarrhoea or increased stoma output early. Inability to drink, dizziness, very little urine, severe pain or blood in stool needs prompt assessment.

Hands, feet and mouth

Pain, cracking, peeling or ulcers that affect normal activities or eating/drinking need advice. Gentle preventive care helps, but does not replace reporting.

Chest symptoms and infection

Stop capecitabine and seek urgent assessment for chest pain. Fever, shaking chills or sudden illness needs immediate oncology contact, even if the temperature is normal.

Treatment-specific effects

Severe toxicity can happen early. Do not assume that tablets are mild treatment or that a normal DPD result makes symptoms safe to ignore.

After a treatment hold

Do not replace doses missed because of side effects. A planned rest week does not by itself mean it is safe to restart. The team checks recovery, hydration, blood results and your previous dose before giving new tablet instructions.

Diarrhoea during the night

Stop capecitabine and contact your treatment team if diarrhoea wakes you during the night. Do not wait for the next clinic or decide the grade yourself. Follow your written bowel-treatment and fluid plan while obtaining advice.

Daily life and other medicines

Make the plan fit your practical needs.

Eating, drinking and activity

Aim for tolerable meals and fluids; contact the team if intake falls. Gentle activity and rest can be adjusted to your energy. Ask for dietetic help if needed.

Medicine and supplement checks

Check new medicines with oncology pharmacy. Warfarin can require closer INR checks; phenytoin levels may need monitoring. Brivudine is an important prohibited interaction.

Family, work and fertility

Ask about infection exposure, work, intimacy, contraception, fertility and any caring responsibilities. Use the medicine-specific duration of precautions given by your team.

Procedures and travel

Plan transport and support around appointments. Ask which symptoms or blood-count problems affect travel and when to contact the service.

Questions worth asking

What if I forget a dose or vomit?

Do not take an extra dose. Take the next planned dose at its usual time unless treatment has been stopped; report repeated vomiting and check with the team if unsure.

Why do I need blood tests if I take tablets at home?

Blood counts, kidney and liver results, together with symptoms, guide whether to continue and at what dose.

Do I need a port or a pump?

No take-home chemotherapy pump is used. Capecitabine is swallowed, so a central line is not required just to give it.

What happens after treatment?

Ask for your review and follow-up plan. Continue reporting symptoms after the last dose; further therapy depends on the treatment aim and response.

Try explaining the plan back: Which days do you take tablets? What do you do if you miss a dose? Which symptoms mean stop tablets and call? Who do you contact out of hours?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Capecitabine · active treatment cycle

Active medicines: Capecitabine

Emetogenic risk: Oral minimal-to-low

Medicine	Dose / route	When
Metoclopramide	10 mg PO up to three times daily as required	Only when needed, as prescribed One option for oral minimal-low risk; check renal function, contraindications, interactions and duration. Do not add all alternatives concurrently.

Additional phase-specific support

- The team agrees a rescue medicine if needed; routine daily anti-sickness prevention is not automatically required for this oral low-risk schedule.
- Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Use gentle skin and mouth care. Painful hands and feet or mouth sores can require a tablet hold and team review; do not continue simply to finish the pack.
- Where prescribed under the capecitabine source: loperamide 4 mg initially then 2 mg after each loose stool, maximum 16 mg/day; follow the written local escalation plan. Do not copy the irinotecan high-dose schedule.
- Swallow capecitabine whole with water and food or within 30 minutes afterwards. Do not crush or cut tablets; missed toxicity doses are not replaced.
- Interrupt immediately for moderate/severe toxicity and obtain oncology advice. Grade 2 or worse dehydration requires interruption until rehydrated and the cause is controlled; do not restart automatically after the planned break.
- Record previous dose reductions before prescribing the next cycle. A table percentage must not automatically re-escalate a toxicity-reduced dose. Partial-dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) starting-dose titration is a separate specialist pathway.
- Confirm blood count, renal and liver profile before every cycle; DPD testing precedes first treatment unless already tested. Plan international normalised ratio (blood-clotting test) (INR) checks if a vitamin K antagonist is used.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth

Capecitabine

Painful ulcers or reduced intake: stop tablets and call.

2 Chest

Capecitabine

New chest pain: stop tablets and seek urgent assessment.

3 Bowel / stoma

Capecitabine

Significant diarrhoea, increased output or dehydration needs prompt advice.

4 Hands / feet

Capecitabine

Pain or peeling affecting daily activity: stop tablets and call.

5 Blood / infection

Capecitabine

Fever, shivers or sudden illness: contact oncology immediately.

6 Hydration / urine

Capecitabine safety

Reduced urine, dizziness or inability to drink needs assessment.

7 Energy / daily tasks

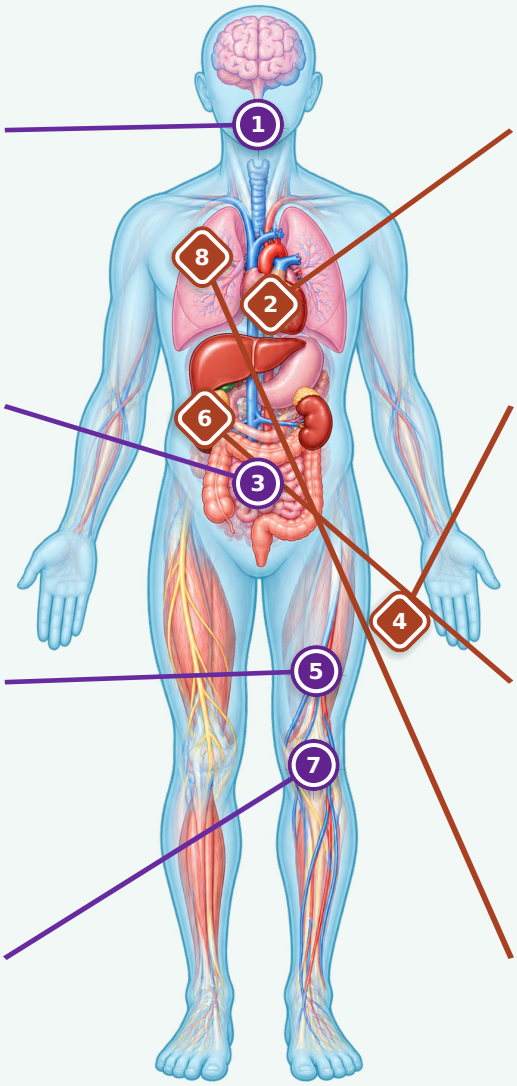
Treatment effects

Report fatigue that prevents usual activities or self-care.

8 Unexpected early illness

Capecitabine

Severe early diarrhoea, mouth soreness or sudden illness needs urgent review.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00216 v8 · official regimen ↗**
   https://healthservice.hse.ie/documents/6558/216_v8_Capecitabine.pdf
2. **eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule) ↗**
   https://www.eviq.org.au/medical-oncology/colorectal/adjuvant-and-neoadjuvant/74-colorectal-adjuvant-capecitabine/patient-information
3. **EMA: Xeloda product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/xeloda-epar-product-information_en.pdf
4. **HSE: if you have side effects of cancer treatment**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
5. **X-ACT: primary trial report**
   https://pubmed.ncbi.nlm.nih.gov/21896539/
6. **CREATE-X: residual invasive breast cancer after neoadjuvant chemotherapy**
   https://pubmed.ncbi.nlm.nih.gov/28564564/
7. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
8. **NCCP Classification V6 (2025), oral minimal-to-low risk Table 5**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf